

Autism Classification Manual

George D. Dranichak

Counselor Education Program, Saint Bonaventure University

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Dr. Demah Payne

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Section 1: Introduction

Autism, at its very core, is a fascinating disability that warrants extensive discussion, not just in academics, but in society at large. Autism Spectrum Disorder (ASD) is defined in the Diagnostic and Statistical Manual of Mental Disorders (5th ed., text rev.; DSM-5-TR) as a neurodevelopmental condition characterized by persistent deficits in social communication and social interaction, along with restricted and repetitive patterns of behavior, interests, or activities. Symptoms must be present in the early developmental period, result in clinically significant impairment, and not be better explained by intellectual disability or global developmental delay (American Psychiatric Association, 2022).

I have a close connection with neurodiversity, and one of my closest friends had a complicated history with it. He revealed to me in 2010 that he had autism, but later, he redacted his statement, claiming it was a misdiagnosis. The initial diagnosis he received was correct, and it turns out it was a family member of his who kept marginalizing him, putting thoughts into his head about the legitimacy of his disability. He came to terms with it in 2018 when he described to me his ability of recalling colors, shapes, and memories in a hyper-specific way, and even saying that his creativity and imagination always felt like a plant that was growing inside his brain. I'll never forget that statement because it really guided my thoughts on the importance of creativity and imagination that I want to foster in my students. His perfectionism led to a laser-focused attention to detail that also inspired me.

Because of my close connection to this friend, I began to think that I had some form of autism. It made sense to me as a kid; I wasn't good with sarcasm, I was fixated on Pokémon, and I didn't like being touched. I would describe my social skills as "masking" because it was hard to engage socially, and faking extraversion was just using my acting skills from the school musicals

in real life. In my teens, I would identify myself with characters like Abed from the TV show, *Community*, a character with clear, defined traits of autism. I would frequently tell my psychiatrist that I felt like I was “on the spectrum” and she explained to me that everyone is on the spectrum to some extent, some just have more symptoms than others. One of the very first papers I researched for this St. Bonaventure Counseling Program was a Summer 2024 lesson on “Navigating Neurodivergence.”

Despite this, I still find it disheartening to see that it is classified as a “disability” at all. Think about it: the word itself has an implication that people like my friend are unable to do certain things. They are “differently abled,” but I must say, from personal experience, he was one of the most optimistic, generous, and uplifting people I’ve ever had the pleasure of coming across in my lifetime. It’s not just him, I’ve had a student that I worked one-on-one with as an IA who had ASD, and his creativity and saccharine personality were incredible to behold. Granted, he was not very skilled socially and had trouble following directions right away, so he did need accommodations, but I was happy to oblige and help him in any way I could.

In doing this classification manual, I hope to feel a stronger connection to my friend and my student I aided, while also deepening my own understanding of ASD. More importantly, this work is an opportunity to bridge the gap between clinical classification and lived experience. I recognize that while ASD is defined through diagnostic criteria, it is truly understood through the individuals who embody it. I aim to present a more balanced and humanizing perspective that supports educators, counselors, and professionals in fostering environments where neurodivergent individuals are not simply accommodated, but genuinely understood and empowered.

Section 2: Applied Framework for Supporting Autism Spectrum Disorder

Social/Emotional needs and measurable goals

Need 1. Social communication and peer interaction.

Goal 1. By the end of the IEP period, during structured peer interactions, [Student Name] will demonstrate appropriate nonverbal communication skills (e.g., eye contact, body orientation) in 80% of opportunities across 3 out of 4 observed sessions, as measured by teacher observation and standardized social skills rating scales.

More information. Difficulties with body language are a core component of social communication deficits in individuals with Autism Spectrum Disorder. Research indicates that children with ASD often experience challenges in both interpreting and expressing nonverbal cues, including facial expressions, gestures, and body orientation, which can significantly impact social interactions. These challenges are linked to broader difficulties in emotion recognition and social cognition, making it harder for individuals with ASD to understand others' intentions and respond appropriately in social contexts. Interventions that explicitly target body awareness and nonverbal communication (e.g., structured social skills training and mindfulness-based approaches) have shown promise in improving these skills and enhancing overall social functioning (Drüsedau et al., 2021).

Need 2. Emotional regulation and coping skills.

Goal 2. By the end of the IEP period, when taught coping strategies, [Student Name] will demonstrate the ability to express frustration using appropriate language or actions with an 80% accuracy rate in four out of five practice situations, as evidenced by behavior charts.

More information. Difficulties with emotional regulation are common among individuals with ASD and can significantly impact behavior, social functioning, and academic

engagement. Research indicates that many students with ASD struggle to identify, manage, and appropriately express emotions, which may lead to frustration, anxiety, or challenging behaviors in classroom settings. These challenges are often addressed through structured, evidence-based interventions that explicitly teach coping strategies, self-management, and emotional awareness. Frameworks such as Positive Behavioral Interventions and Supports (PBIS) emphasize the importance of teaching and reinforcing adaptive emotional and behavioral responses, helping students replace maladaptive behaviors with appropriate, functional alternatives (Watkins et al., 2024).

Common accommodations and modifications

Within a Multi-Tiered System of Supports (MTSS), interventions and accommodations are implemented across increasing levels of intensity and are guided by data-based decision-making to meet students' academic, behavioral, and social-emotional needs (Rangel-Pacheco et al., 2024).

MTSS Tier 1.

- Use visual supports (e.g., schedules, graphic organizers) and clearly structured routines to improve predictability and understanding of expectations.
- Provide frequent positive reinforcement and explicit instruction of behavioral and social expectations to support engagement and appropriate classroom behavior.

MTSS Tier 2.

- Implement social skills training and peer-mediated interventions to improve social interaction and communication with peers.
- Use self-monitoring strategies, task analysis, and structured check-in/check-out systems to support independence and on-task behavior.

MTSS Tier 3.

- Conduct a Functional Behavior Assessment (FBA) and implement individualized Behavior Intervention Plans (BIPs) to address challenging behaviors and teach appropriate replacement skills.
- Provide individualized supports such as Functional Communication Training (FCT), cognitive-behavioral strategies, or augmentative communication systems to address complex behavioral and emotional needs.

Common assessments and testing

- **Autism Diagnostic Observation Schedule, Second Edition (ADOS-2):** A standardized, semi-structured observational assessment used to evaluate social communication, interaction, and restricted/repetitive behaviors associated with ASD. It is widely considered a “gold-standard” tool for ASD assessment; however, it should not be used in isolation, as research shows that while it demonstrates high sensitivity, it may produce false positives in individuals with other psychiatric conditions, particularly those with overlapping social communication difficulties (Maddox et al., 2017).
- **Autism Diagnostic Interview–Revised (ADI-R):** A standardized, semi-structured caregiver interview used to gather detailed developmental history related to social communication, play, and restricted or repetitive behaviors associated with ASD. The ADI-R systematically evaluates symptoms across early development and is widely used in both clinical and research settings; however, research indicates that while it demonstrates adequate diagnostic validity, its accuracy can vary depending on language ability and population, reinforcing the need to use it alongside other assessment tools and clinical judgment (Lampinen et al., 2025).

Common behavior concerns and strategies

Behavior Concerns

- Hyperactive, impulsive, and/or inattentive behavior
- Anxiety, stress, or excessive worry
- Gets upset by minor changes

(CDC, 2024)

Strategies

- Teach students to identify emotions
- Coping with techniques (breathing, breaks, etc.)
- Reduce sensory overload (noise, lights, etc.)

(Mazefsky et al., 2013)

Expert tips and recommendations

- **Incorporate interventions that target self-perception and social awareness:**

Structured approaches that focus on body awareness, mindfulness, and nonverbal communication can improve social functioning and emotional understanding in students with ASD (Drüsedau et al., 2021).

- **Utilize evidence-based, structured interventions for social and behavioral skills:**

Practices such as explicit instruction, reinforcement, and peer-mediated strategies are effective in improving social communication and adaptive behavior in school settings (Watkins et al., 2024).

- **Implement supports within a Multi-Tiered System of Supports (MTSS):** Effective intervention requires a tiered approach that includes universal, targeted, and intensive

supports guided by ongoing data collection and progress monitoring (Rangel-Pacheco et al., 2024).

- **Create validating and supportive environments:** Providing a supportive environment that acknowledges both challenges and strengths can improve emotional regulation and reduce frustration in students with ASD (Mazefsky et al., 2013).
- **Prioritize emotional regulation as a central focus of intervention:** Many behavioral challenges in ASD stem from difficulties with emotion regulation; teaching coping strategies, emotional awareness, and flexibility can significantly improve outcomes (Mazefsky et al., 2013).
- **Promote neurodiversity-affirming and ethically informed practices:** Interventions should respect student autonomy, support identity development, and avoid prioritizing conformity to societal norms over individual well-being (Wilkenfeld & McCarthy, 2020).

Ethical and legal issues

Ethical and legal considerations play a critical role in the assessment and treatment of ASD, particularly when interventions involve modifying behavior, communication, and social functioning. While approaches such as Applied Behavior Analysis (ABA) are widely used and supported within educational and clinical settings, they have also been the subject of ongoing ethical debate. Concerns have been raised regarding issues of autonomy, identity development, potential harm, and the extent to which interventions prioritize societal norms over individual well-being. As a result, it is essential for educators, counselors, and practitioners to critically evaluate both the benefits and limitations of commonly used interventions, ensuring that practices remain ethical, evidence-based, and responsive to the needs and rights of neurodivergent individuals (Wilkenfeld & McCarthy, 2020).

- **Autonomy:** Students with ASD have autonomy-related interests, and interventions should respect their preferences, communication styles, and developing sense of self, even when adults hold decision-making authority.
- **Identity Development:** Interventions that focus heavily on behavior modification may influence how students develop their identity, particularly if they discourage natural behaviors or personal interests.
- **Nonmaleficence:** Intensive or highly structured interventions may lead to stress or emotional fatigue, highlighting the importance of monitoring student well-being alongside behavioral progress.
- **Justice and Inclusion:** Ethical concerns arise when interventions prioritize conformity to societal norms over individual well-being, highlighting the need for inclusive environments that support neurodiversity rather than requiring students to bear the full burden of adaptation.
- **Informed Consent and Parental Rights:** Parents have the legal right to participate in decision-making regarding evaluations, placements, and interventions, including accepting or declining specific services.

Appendix of resources for more information

- **Centers for Disease Control and Prevention (CDC) – Autism Spectrum Disorder (ASD)**
 - **URL:** <https://www.cdc.gov/autism>
 - Provides easy-to-understand information on signs, symptoms, early development, and support strategies for children with ASD.
- **Autistic Self Advocacy Network (ASAN)**

- URL: <https://autisticadvocacy.org>
- Provides neurodiversity-affirming resources written by autistic individuals, focusing on self-advocacy, rights, and identity.
- **National Autism Center – Evidence-Based Practice Guidelines**
 - URL: <https://www.nationalautismcenter.org>
 - Summarizes research-backed interventions in a way that is accessible to parents and educators.
- **Organization for Autism Research – Parent Guides**
 - URL: <https://researchautism.org>
 - Provides practical, easy-to-read guides for parents on topics such as behavior, education, and transition planning.
- **Autism Society**
 - URL: <https://www.autism-society.org>
 - Provides accessible, family-friendly information on diagnosis, services, advocacy, and community support for individuals with ASD.

Section 3: Reflection

In doing this paper, I recognized that students with disabilities, including those with ASD, often navigate educational environments that are not fully designed to meet their diverse needs. Physical disabilities are often met with clear, observable accommodations, such as ramps or accessible infrastructure; however, students with mental or developmental conditions may not receive equally visible or consistently implemented supports, highlighting a critical disparity in how different disabilities are addressed within educational systems. I recognize that special education administration is a complex and demanding responsibility, particularly when balancing legal requirements, resource limitations, and the evolving, diverse needs of students. However, legislation such as the Individuals with Disabilities Education Act (IDEA) and the guarantee of a Free Appropriate Public Education (FAPE) have significantly advanced the expectation that students with disabilities receive individualized and appropriate accommodations. Despite this progress, there remains a need for more accessible, research-based interventions that can be effectively implemented within school settings. Research highlights the importance of implementing structured, data-driven interventions within frameworks such as MTSS to better meet diverse student needs (Rangel-Pacheco et al., 2024). A key limitation is the lack of accessible, comprehensive research guiding practical intervention in school settings.

Common strategies such as sensory tools or breaks don't feel like enough to me. Many commonly used strategies may not fully address underlying emotional regulation challenges, which are central to behavioral difficulties in students with ASD, for example (Mazefsky et al., 2013). There is a clear gap between what is currently implemented in schools and what may be possible through continued research and innovation in intervention practices.

That's why, as a school counselor, I would take an active role in bridging this gap by seeking out and implementing evidence-based strategies that extend beyond surface-level accommodations. This includes collaborating with multidisciplinary teams, staying informed on current research, and advocating for interventions that are both effective and feasible within school guidelines. By doing so, I aim to move beyond simply meeting minimum requirements and instead contribute to creating a more responsive and supportive environment for students with disabilities.

One article that stood out was Wilkenfeld and McCarthy's (2020) critique of Applied Behavior Analysis (ABA), which raised important ethical concerns regarding current intervention practices. One thing I took away from it, albeit indirectly, was the concept of "minimal compliance," doing something to satisfy a criterion or obligation, without tackling the full scope of issues. From an ethical perspective, simply meeting legal requirements is not sufficient; educators must strive to implement practices that meaningfully support student well-being and respect individual differences (Wilkenfeld & McCarthy, 2020).

In animation, there's a concept called "bumping the lamp," taken from the film, *Who Framed Roger Rabbit?* Rather than relying on static lighting, animators chose to take on the added challenge of dynamically adjusting light and shadow as a lamp moved within a scene, enhancing realism and depth. At a time when animation was entirely hand-drawn, this level of detail required intentional effort and precision beyond what was strictly necessary. All this is to say, I feel like working with students with disabilities creates two paths: you could either follow minimal compliance and just do what's necessary to get by, or you could bump the lamp and make a true difference. In my role as a school counselor, I strive to choose the latter, recognizing that meaningful support often lies not in what is required, but in what is possible.

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** Artificial intelligence (AI) tools were used in a limited and supportive capacity during the development of this paper. Specifically, AI was utilized to assist in organizing information, summarizing scholarly sources, and refining the clarity and structure of written content. All ideas, interpretations, and final written material were reviewed, edited, and verified by me to ensure accuracy and alignment with course expectations. AI was not used to generate original arguments or replace engagement with primary academic sources. All cited materials were independently selected, evaluated, and incorporated by me in accordance with university academic integrity standards.*